

DIFFICULTY SLEEPING

Assess the patient with difficulty sleeping

- Confirm that the patient really is getting insufficient sleep. Adults need on average 6-8 hours sleep per night. This decreases with age.
- Determine the type of sleep difficulty: waking too early or frequently, difficulty falling asleep, insufficient sleep.

Exclude medical problems:

- Ask about pain, difficulty breathing, urinary problems. See relevant symptom pages. If persistent snoring, consider **obstructive sleep apnoea** ↗ 38. If restless legs, refer to doctor for further assessment.
- If patient has a chronic condition, give routine care.
- If pulse ≥ 100 , weight loss, palpitations, tremor, dislike of hot weather or thyroid enlargement, check TSH. If abnormal, refer to doctor.

Review medication:

- Over-the-counter decongestants, salbutamol, fluoxetine, efavirenz can cause sleep problems. Discuss with doctor.
- Reassure patient that difficulty sleeping from efavirenz is usually self-limiting and resolves within 6 weeks on ART. If severe or > 6 weeks, discuss with doctor.

Assess alcohol/drug use:

- In the past year, has patient: 1) drunk ≥ 4 drinks¹/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.

Screen for possible stressors and mental health problem:

- If stress or anxiety ↗ 86.
- Has the patient ever had a bad experience that is causing nightmares, flashbacks, avoidance of people/situations, jumpiness or a feeling of detachment? If yes ↗ 88.
- In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143.
- If abnormal thoughts or behaviour ↗ 85.
- If for at least 6 months ≥ 1 of: memory problems, disorientation, language difficulty, less able to cope with daily activities and work/social function: consider dementia ↗ 148.

Ask about menopausal symptoms:

- If woman > 40 years ask about menopausal symptoms: hot flushes, night sweats, vaginal dryness, mood changes and sexual problems ↗ 169.

Advise the patient with difficulty sleeping

- Encourage patient to adopt sensible sleep habits. These often help to resolve a sleep problem without the use of sedatives.
 - Get regular exercise.
 - Avoid caffeine (coffee, tea, sweetened fizzy drinks), alcohol and smoking for several hours before bedtime.
 - Avoid day-time napping.
 - Encourage routine: get up at the same time each day (even if tired) and go to bed the same time every evening.
 - Allow time to unwind/relax before bed.
 - Use bed only for sleeping and sex. Spend only 6-8 hours a night in bed.
 - Once in bed, avoid clock-watching. If not asleep after 20 minutes, do a low energy activity (read a book, walk around house). Once tired, return to bed.
 - Keep a sleep diary. Review this at each visit.
- Review the patient regularly. A good relationship between practitioner and patient can help.

Refer patient for further assessment if problems with daytime functioning, daytime sleepiness, irritability, anxiety or headaches that do not get better with 1 month of sensible sleep habits.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.